

2025 American Diabetes Association (ADA) Clinical Guidelines

Section 9: Pharmacologic Approaches to Glycemic Treatment

Metformin remains the preferred initial pharmacologic agent for the treatment of type 2 diabetes. However, the 2025 guidelines emphasize that early combination therapy should be considered in more patients at treatment initiation to extend the time to treatment failure and preserve beta-cell function.

For patients with established atherosclerotic cardiovascular disease (ASCVD) or indicators of high ASCVD risk (such as patients >55 years of age with coronary, carotid or lower-extremity artery stenosis >50% or left ventricular hypertrophy), a sodium-glucose cotransporter 2 (SGLT2) inhibitor or glucagon-like peptide 1 (GLP-1) receptor agonist with demonstrated cardiovascular disease benefit is highly recommended.

Section 10: Cardiovascular Disease and Risk Management

Blood pressure should be measured at every routine clinical visit. Patients with blood pressure $\geq 140/90$ mmHg should have blood pressure confirmed using multiple readings, including measurements on a separate day, to diagnose hypertension. All hypertensive patients with diabetes should monitor their blood pressure at home. Treatment for hypertension should include lifestyle interventions and pharmacologic therapy.

Section 11: Chronic Kidney Disease and Risk Management

At least once a year, assess urinary albumin (e.g., spot urinary albumin-to-creatinine ratio) and estimated glomerular filtration rate (eGFR) in patients with type 1 diabetes with duration of ≥ 5 years and in all patients with type 2 diabetes regardless of treatment. Patients with urinary albumin ≥ 300 mg/g and eGFR ≥ 20 should be treated with an SGLT2 inhibitor to reduce CKD progression and cardiovascular events.